



February 23, 2026

U.S. Department of Health and Human Services
Office of the Deputy Secretary
Hubert H. Humphrey Building
200 Independence Avenue, S.W.
Washington, D.C. 20201.

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Docket Number: HHS-ONC-2026-0001-0001

Submitted electronically at: <https://www.regulations.gov/document/HHS-ONC-2026-0001-0001>

Dear Deputy Secretary O'Neill:

As nursing stakeholders, the Alliance for Nursing Informatics (ANI) is pleased to offer comments on **“Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.”**

[The Alliance for Nursing Informatics](#) (ANI), cosponsored by the American Medical Informatics Association (AMIA) and the Healthcare Information and Management Systems Society (HIMSS), advances nursing informatics leadership, practice, education, policy, and research through a unified voice of nursing informatics organizations. We transform health and healthcare through nursing informatics and innovation. ANI is a collaboration of organizations representing more than 25,000 nurse informaticists and bringing together 29 distinct nursing informatics groups globally. ANI crosses academia, practice, industry, and nursing specialty boundaries and collaborates with more than 4 million nurses in practice today.

Below, we provide responses to each of the questions included in the RFI. In addition, we offer the broad suggestion that HHS and policymakers consider conceptualizing AI through the lens of care delivery; it should be layered on top of areas where evidence is fragmented or weak, but not replace the honed clinical judgment of expert clinicians or the therapeutic relationship they establish with patients and families. Through this lens, we highlight that nurses provide continuous care for hospitalized patients and are the principal documenters of the clinical data that AI models should be developed, trained, validated, and deployed using this data- a current limitation of the tools in development. Nurses are also at the forefront of care delivery, interfacing with patients and families more than any other health profession, and will thus witness the effects of AI on patient care most clearly. Additionally, we highlight that this frontline positioning exposes nurses to professional liability; nurses may be the first to detect

AI-related errors and may face documentation, accountability, and liability questions when AI meaningfully informs decisions. As such, their voices must be centered in AI development, testing, and deployment decisions. Without nursing engagement, AI tools risk being ineffective, burdensome, or even harmful to patient care.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

A significant barrier includes the substantial return on investment requirements and the level of effort needed for each individual health system to implement and validate AI tools.¹ The burden of implementation, testing, and outcome measurement is considerable and often falls disproportionately on already resource-constrained clinical teams. Additionally, there is insufficient cross-collaboration infrastructure to effectively vet, test, and demonstrate AI tools across different care settings before widespread adoption. Health systems are often operating in silos, each independently validating tools that have already been tested elsewhere, creating inefficiency and slowing the pace of innovation. The lack of standardized evaluation frameworks means that promising AI tools may struggle to gain traction even when they have demonstrated efficacy in specific contexts. We also note developer and implementer access to high-quality EHR data as a barrier; they often have limited access to representative data, and there is variability in documentation/data quality and constraints on using data for development/validation (privacy, governance, contracting). This slows innovation and increases the local validation burden. Finally, interoperability is a barrier. Inconsistent data models/terminologies, variable EHR configurations, and limited standardized interfaces make it difficult to deploy AI tools across sites and to compare outcomes, contributing to duplicated implementation effort.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

We suggest that HHS regulations and policies be examined to ensure they facilitate safe care transitions both within settings (such as from ICU to step-down to home) and across settings (such as from acute care to post-acute or long-term care), for example, through AI-generated handoff summaries and discharge planning. The focus on USCDI data element sets provides important guidance, yet we need policies that enable this minimum data set to flow seamlessly across the continuum of care. Current fragmentation in data sharing creates barriers to AI tools that could support care coordination and prevent adverse events during transitions. Further, we urge caution around payment policies or programs that reward or incentivize the adoption of evidence-based AI tools without commensurate investments in technical infrastructure and interoperability capabilities across care settings nationally. Presently, many health systems and care delivery organizations- particularly rural hospitals, critical access hospitals, and long-term care, lack basic technical infrastructure to adopt AI, and such programs might exacerbate differences in resources and patient outcomes.¹

¹ Poon EG, Lemak CH, Rojas JC, et al. Adoption of artificial intelligence in healthcare: survey of health system priorities, successes, and challenges. *J Am Med Inform Assoc.* 2025;32(7):1093-1100.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures. What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

A critical implementation issue that HHS must address is the lack of general education and competency development about AI across the clinical workforce. Healthcare professionals lack a foundational understanding of how to use AI tools safely, interpret their outputs appropriately, and recognize their limitations. This knowledge gap creates significant patient safety risks and liability concerns.² HHS should play a leadership role in establishing baseline AI competency standards for clinical professionals and supporting educational initiatives to build AI literacy across disciplines. This includes understanding when AI recommendations should be questioned, how bias can manifest in AI outputs, and what data quality issues might affect model performance. The rapid pace of AI deployment has far outstripped workforce education, and HHS has a unique role in setting standards and supporting training infrastructure that ensures clinicians can be safe, effective users of AI tools. Another critical issue is model transparency. Clinicians and end users must be able to understand how AI tools generate their recommendations and suggestions in order to appropriately evaluate and act on them. Without sufficient transparency into model reasoning and decision-making processes, clinicians cannot fulfill their professional obligation to exercise independent clinical judgment, and patients cannot meaningfully participate in shared decision-making about their care. HHS should encourage or require transparency standards that enable clinicians to understand the basis for AI-generated outputs, including the data sources, logic, and confidence levels underlying recommendations, so that AI serves as a tool that supports informed clinical reasoning and the safest delivery of care. Specific aspects of concerns include: (1) auditability (accessible logs of AI inputs/outputs, model versioning, and user actions), (2) user verification requirements (expectations that clinicians verify AI outputs before acting), (3) mechanisms to correct or override AI-generated content with a documented rationale, and (4) clarity on standard-of-care implications and liability allocation across vendor, health system, and clinician when AI meaningfully informs decisions.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful?

The field of AI evaluation is in nascent stages and, while many have proposed frameworks, benchmarking methods, and metrics, there are no agreed-upon standards. HHS should support rigorous evaluation of AI tools through grant mechanisms that enable both pre-deployment testing and post-deployment monitoring. Grants provide the most flexible and impactful mechanism for supporting systematic evaluation research because they allow partnerships between academia, practice, and industry to develop comprehensive evaluation frameworks that include workflow analysis, human factors assessment, and real-world effectiveness studies. We note that biomedical informatics professionals have been conducting this research for decades and are

² Galatzan B, Johnson EA, Reading Turchioe M, et al. Regulating at the AI frontier: The collision of policy, regulation, and nursing practice. *Journal of Nursing Regulation*. 2025;16(3): 207-215.

already well equipped with the skills, knowledge, and resources to conduct such work. Furthermore, we feel that evaluation methods must include human-centered design principles and incorporate feedback from the clinicians who will actually use these tools in practice. Examples include robustness testing across workflow conditions and data quality variation, post-deployment drift monitoring, and human factors evaluation (e.g., usability testing and cognitive walkthroughs). Too often, AI tools are developed without meaningful clinician input and fail to integrate smoothly into existing workflows, creating new burdens rather than alleviating them. Grant-funded evaluation studies should be required to include multiple distinct clinical settings and populations to ensure that AI tools perform equally well across different contexts and patient groups.

5. How can HHS best support private sector activities to promote innovative and effective AI use in clinical care?

Currently, many AI tools are developed by technology companies, computer scientists, and engineers with limited clinical input, resulting in products that may be technically sophisticated but clinically impractical or unsafe.³ HHS should consider establishing requirements that clinicians, including nurses, who are the largest health profession, must be meaningfully included in all stages of AI development and testing. HHS could operationalize this by embedding clinician partnership expectations into certification/accreditation criteria, as well as through HHS funding mechanisms (grants, pilots, demonstrations), CMS program participation requirements and incentives, and ONC/ASTP implementation guidance tied to interoperability/health IT policy. Naming these levers will make the recommendation actionable and enforceable. This requirement would not position these clinicians as advisors but rather as partners in defining requirements, testing prototypes, and validating real-world performance. HHS can support private sector innovation by creating certification or accreditation pathways that recognize and reward developers who engage in robust clinician co-design and testing processes. This would create market incentives for better development practices while ensuring that AI tools are truly fit for purpose in clinical care. Also, we note that many of the clinical health information technologies are developed and produced by one very large health record vendor that uses a similar design and testing cycle for each innovation. This requirement would discourage "smaller" innovators from embedding design flaws in a large group of AI products.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

There is significant promise in the generative, ambient, and agentic AI space to assist with nursing care and documentation efficiencies. Ambient documentation has demonstrated the ability to significantly improve clinician experience and patient satisfaction, and has already been deployed widely for physicians.⁴ Yet nursing practice

³ Reading Turchioe M, Pepingco C, Ronquillo C, et al. Education, empowerment, and elevating nursing voices: Nursing informatics leaders' perspectives on the path forward with artificial intelligence in nursing. *Nurs Outlook*. 2025;73(5):102484.

⁴ You JG, Dbouk RH, Landman A, et al. Ambient Documentation Technology in Clinician Experience of Documentation Burden and Burnout. *JAMA network open*. 2025;8(8).

has largely been left out of efforts to extend these capabilities/beneficial outcomes.⁵ Given the nationwide shortage of nurses who provide 24/7 continuous care and are constantly documenting the clinical data that AI models train on, there should be urgent, focused attention on alleviating documentation burden so bedside nurses can concentrate on the hands-on, critical elements of patient care that require human judgment, empathy, and skill. The potential impact on nurse retention, job satisfaction, and patient safety makes this a critical priority area.

7. Which roles, decision makers, or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

The most influential decision-makers for AI adoption include practice councils, security/risk/compliance committees, and AI governance bodies where they exist. However, a primary administrative hurdle is inadequate staff education and limited understanding among leadership of both the risks and the return on investment associated with AI tools. Organizations often lack frameworks for systematically evaluating AI tools or established processes for scaling successful pilots to enterprise-wide deployment. Practice councils, which include frontline clinicians, are critical for adoption success but often lack representation in early AI procurement and development decisions. Nurses must be at the table during these discussions, not merely informed after technology decisions have been made. The CMS Health Technology Ecosystem initiative, which includes over 600 industry partners developing AI conversational agents, highlights the urgent need for nursing representation in technology governance. We encourage HHS to engage nurses meaningfully in oversight groups for initiatives like the new CMS App Store for patient applications, ensuring that the clinical perspective of continuous care providers shapes these important decisions.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Standardized data elements that flow seamlessly across care settings and EHR platforms would enable AI tools to function effectively regardless of the underlying technology infrastructure. One notable example is Epic's Cosmos platform, which is being leveraged across Epic's customer base to enable large-scale data analysis and AI model development. However, interoperability must extend beyond single-vendor ecosystems. This is particularly critical for AI applications supporting care transitions and care coordination, where patients frequently move between systems using different EHR platforms. We note that HHS should consider the HTI-5 Proposed Rule carefully, on which we also plan to offer comments, as it includes significant deregulatory actions that may reduce interoperability. Finally, data standards that support nursing documentation and capture the unique contributions of nursing care would be particularly valuable for training AI models that support nursing workflows.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and

⁵ Van Tiem J, Cramer E, Iverson C, et al. Listening to the note: clinician perspectives on ambient artificial intelligence scribes in medical documentation. *J Am Med Inform Assoc.* 2025.

caregivers have related to the adoption and use of AI in clinical care?

We encourage a review of the numerous studies that have explored patients' and caregivers' perspectives on AI across contexts and care settings.⁶ For example, patients have concerns about data confidentiality and how AI will impact the cost of care, especially in mental health contexts.⁷ Patients typically fear AI disrupting the relationship they have with their care team, and have significant concerns about accuracy and explainability. From the patient perspective, there is promise in AI's ability to provide self-help resources and serve as a last chance at hope when AI tools might identify insights or patterns that time-pressed medical providers may have missed because they lack the capacity to dig deeply into literature or review comparable case studies. This is underscored by recent research showing that 1 in 4 active users of OpenAI's ChatGPT prompt about medical questions weekly, and other research showing that 60% of adults age 50 and older have used it for medical advice at least once.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

HHS should prioritize: (1) investments in biomedical informatics research, (2) mechanisms for academic-industry partnerships, and (3) research and investments on human factors consideration with AI implementations, especially ambient AI applications and especially in nursing practice. First, HHS should fund research on AI evaluation methodologies that can be systematically applied across different clinical contexts to determine which tools genuinely improve care quality and reduce costs versus those that simply add complexity. The literature currently lacks comprehensive approaches to evaluating the costs, benefits, and tradeoffs of using AI in clinical care, particularly from the nursing perspective. Research investments should also explore how to scale successful AI implementations across different healthcare settings and how to ensure AI tools perform equitably across diverse patient populations. Second, these research priorities will likely require partnerships between companies developing scalable AI tools and the healthcare delivery organizations containing the patient populations and clinical expertise necessary to evaluate these tools in a rigorous, generalizable manner. Therefore, partnerships between such organizations could accelerate learning and dissemination of best practices, but require grant or contract funding. Embedding nursing leadership in these collaborations is essential to ensure that implementation strategies are clinically feasible, patient-centered, and aligned with frontline care realities. Third, research is critically needed on human factors considerations such as the cognitive burden and workflow impact of clinical AI, including effects on clinician mental workload, attention, task switching, and documentation load, and how interface design and escalation pathways can reduce cognitive overload and automation bias. Moreover, we note that ambient documentation is rapidly being adopted and integrated into care, but the evidence base lags behind. There is also currently limited published evidence on how ambient AI tools specifically affect nursing care delivery, despite the significant potential impact on the largest segment of the clinical workforce. Research should examine current data about ambient AI and nursing,

⁶ Osnat B. Patient perspectives on artificial intelligence in healthcare: A global scoping review of benefits, ethical concerns, and implementation strategies. *Int J Med Inform.* 2025;203:106007.

⁷ Benda NC, Harkins SE, Hermann A, et al. A Cross-sectional Study of Patient Perspectives on Artificial Intelligence: A Comparison of Somatic Versus Mental Health Care. *J Gen Intern Med.* 2025;40(4):977-979.

including implementation barriers, usability challenges, and best practices for deployment.

In conclusion, we encourage HHS to adopt the framing that AI is a tool in service of care delivery and transitions, not an end in itself. Through this patient-centered lens, it is clear that nurses are not peripheral stakeholders in AI-enabled care but rather essential to its success. ANI strongly advocates for HHS to recognize that successful AI adoption in clinical care requires meaningful nursing engagement at every level, from development and testing through implementation and evaluation. With thoughtful policy that centers patients' and clinicians' needs and ensures adequate education, oversight, and evaluation, AI has tremendous potential to address the healthcare workforce crisis while improving outcomes for patients and communities across America.

Sincerely,



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The [Alliance for Nursing Informatics](#) (ANI), cosponsored by AMIA & HIMSS, advances nursing informatics leadership, practice, education, policy, and research through a unified voice of nursing informatics organizations. We transform health and healthcare through nursing informatics and innovation. ANI is a collaboration of organizations representing more than 25,000 nurse informaticists and bringing together 29 distinct nursing informatics groups globally. ANI crosses academia, practice, industry, and nursing specialty boundaries and collaborates with more than 4 million nurses in practice today. [Contact ANI.](#)